



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : عيبر محمد مهدي ابراهيم الحويطي

المحافظة : القليوبية الحي / المدينة : القناطر الخيرية العنوان : 00

رقم البطاقة : 28712121404422 رقم التليفون : 01062260390 تاريخ تسجيل الإستمارة : 23-09-2026

العمر (تاريخ الميلاد) : 12-12-1987 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح PET/CT للجسم [8530] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني	الوظيفة: -----	التوقيع: -----
الاسم: د/ندى حسن	الوظيفة: -----	التوقيع: -----
الاسم: د/نرمين رمضان	الوظيفة: -----	التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: عيبر محمد مهدي ابراهيم الجويط National ID: 28712121404422	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

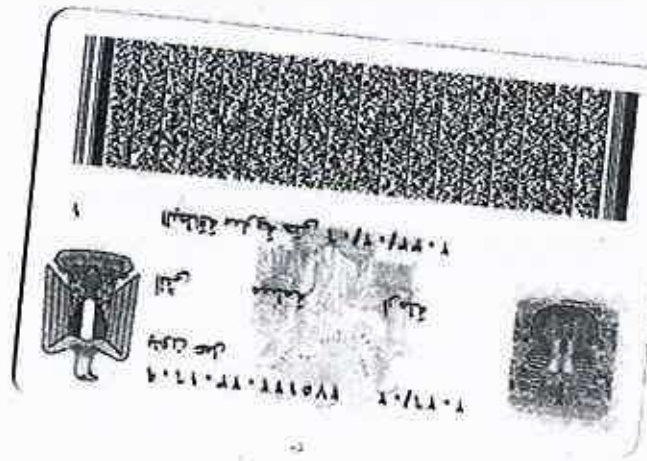
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Whole-body PET/CT scan.
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



2312--2011.





MDT FORM

CC / E / 2027 G

Name	عبيد محمد موري	MRN	195
Age	38 سنة	ID	
Menopausal status	Pre	Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:

ER:

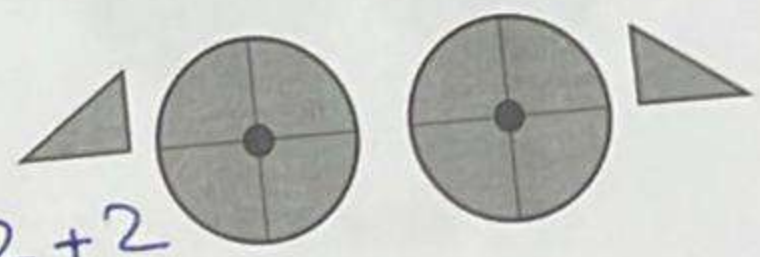
PR:

HER 2:

Ki67:

R

L



TNM

Sonomammography:

2021 → ER 7/8 PR 8/8 her 12 + 2
R+ MRM

Mets Workup:

Adj: PORTH + HT

2024 → HFL

BX: ER 3/8 - PR 0/8 - her 12 -ve

Recommendations:

For: Xeloda single agent
+ Zoladex 10.8. 13m.

MDT OUTCOME & PLAN

- ① Re-do Neck up + 1-bipy for clinically detected contralateral Cervical LN.
- ② Monthly shift to Xeloda + Zoladex (initially HER+ve).

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Handwritten signatures and dates in the MDT outcome section.					

Patient Name : الأستاذ / عبير محمد مهدى إبراهيم الحويطى
Patient No. : 28712121404422
Path No. : 1251/26
Sex : Female
Age : 38 Y
Referring Doc :
Request No. : 13006210
Request Date : 30/04/2026
Verification Date : 12/05/2026
Financial Category : جوسف روسى - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Known case of breast cancer with suspicious cervical lymph node
Nature of Specimen: Tru-cut biopsy, collected at 30/04/2026, 12:40 pm, cold ischemia time: 20 seconds.

Gross Examination:

- Multiple grayish white tissue cores largest 0.5 cm, totally submitted.

Microscopic Examination:

- Histopathological examination of left cervical lymph node showed metastatic deposits of invasive duct carcinoma grade 3 with positive lymphovascular emboli (in capsule).

Diagnosis:

Left cervical lymph node, Tru-cut biopsy:
Positive for metastatic invasive duct carcinoma, Grade 3.
For further IHC (GATA-3, ER, PR and Her2).

Signature
Dr. Walaa Ibrahim Ghoneim

Verified by :
Dr Maha Guimie

Printing Date : 17/05/2026 08:04:14

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / عيبر محمد مهدي ابراهيم الحويطي	Request No.	: 13006210
Patient No.	: 28712121404422	Request Date	: 30/04/2026
Path No.	: 1251/26	Verification Date	: 12/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left cervical lymph node "diagnosed as metastatic invasive duct carcinoma grade 3".
Nature of Specimen: Tru-biopsy, collected at 30/04/2026, at 12:40 pm.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- GATA-3 clone (L50-823), ER clone (1E2), PR clone (SP1), Her2 clone (4B5).

Results:

- * IPT (All controls showed appropriate reactivity):
- GATA-3: Diffuse strong positive.
- ER: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).

Conclusion:

Left cervical lymph node, Tru-cut biopsy:

IPT:

- ER: Positive (score 7/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+). For SISH.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and

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Patient Name	الأستاذة / عيبر محمد مهدي إبراهيم الحويطي	Request No.	13006210
Patient No.	28712121404422	Request Date	30/04/2026
Path No.	1251/26	Verification Date	12/05/2026
Sex	Female	Financial Category	جوستاف روسي - نفقة الدولة
Age	38 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim

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Dr Maha Guimie

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Page 2 of 2

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Age : 38 Y	Department : Out Patient
Refereing Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left cervical lymph node "diagnosed as metastatic invasive duct carcinoma grade 3".

Nature of Specimen: Tru-biopsy, collected at 30/04/2026, at 12:40 pm.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- Her2 clone (4B5), & SISH.

Results:

- * IPT (All controls showed appropriate reactivity):
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
- (Average Her2 copy No.: (2.05), Her2/CEP17 ratio: (0.93)

Conclusion:

Left breast cancer, Tru-cut biopsy:

IPT:

- Her2: Equivocal (score 2+).
- SISH: Not amplified (Group V).

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and

Verified by :

Dr Maha Guimie



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Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ibrahim Ghoneim

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Patient's Name: Abeer Mohammed Mahdi Ibrahim
Date: 2026-07-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATION OF THE NECK, CHEST, ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- No appreciable changes seen as regards to the previously noted bilateral deep cervical LNs and the largest on the right side still measures about 14mm.
- Evident right mastectomy with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed.
- Normal left breast and both axillae.
- Stability of the previously-noted left perivascular lymph nodes with no other sizable mediastinal lymph nodes noted.
- Hyperinflated both lungs.
- Stability of the mild subpleural reticulations seen in the anterior segment of right upper pulmonary lobe and a thin linear atelectatic band seen in the left lower pulmonary lobe.
- No sizable newly-developed abnormally-enhancing pulmonary nodules, patchy areas of consolidations or foci of calcifications noted.
- No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Normal bones of the thoracic cage.
- Mildly increased sized liver, measuring about 17.3 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- Vanished the previously-seen tiny non-enhancing lesion in segment IVb.
- No sizable newly-developed abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen, measuring about 11.4 cm. No focal lesions seen inside.

Patient's Name: Abeer Mohammed Mahdi Ibrahim
Date: 2026-07-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Normal CT appearance of the pancreas, functioning both kidneys(with persistence of right lower caliceal stone,measuring about 9.2mm with CT density of 1000HUs), both suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.
- No appreciable changes as regards to the tiny sclerotic bony lesions previously-seen in the right iliac and to lesser extent in the left pubic bone.

IMPRESSIONS:

A status follow up since the lastly done study at 19/04/2026 with the current study shows:

- Evident right mastectomy with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed.
- Vanished the previously-seen tiny non-enhancing lesion in segment 1Vb of the left hepatic lobe,denoting a complete resolution.
- No appreciable changes seen as regards to the previously noted bilateral deep cervical LNs and the largest on the right side still measures about 14mm,denoting a stable course.
- Stability of the previously-noted left perivascular mediastinal lymph nodes, the mild subpleural reticulations in the anterior segment of right upper pulmonary lobe and a thin linear atelectatic band in the left lower pulmonary lobe and no appreciable changes as regards to the tiny sclerotic bony lesions previously-seen in the right iliac and to lesser extent in the left pubic bone,denoting their stable course.

Prof/Hanan El-Ahwal.

Patient's Name: Abeer Mohammed Mahdi Ibrahim
Date: 2026-07-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE CT EXAMINATIUN OF THE CHEST ,ABDOMEN AND PELVIS WITH
CONTRAST ADMINISTRATION REVEALED:-**

- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Mosaic appearance of the scanned bones with osteolysis/sclerotic foci scattering in the scanned spine with wedge collapse of DV8 and to lesser extent DV11,and DV12.

Patient's Name: Abeer Mohammed Mahdi Ibrahim
Date: 2026-07-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Mildly enlarged liver,measuring about 17.5 cm, showing a homogenous parenchymal attenuationand e

Name : Abeer Mohammed Mahdi Ibrahim
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 22-06-2026 06:07
Medical No. : 399388

Tc-99m MDP Whole-Body Bone Scintigraphy

Clinical History & Indication

A 37-year-old female patient with a history of right breast cancer presented for routine follow-up staging and skeletal surveillance. Active complaints of specific bony aches are noted.

Technique

Following the intravenous administration of Tc-99m MDP via an indwelling venous access port (Port-A-Cath), anterior and posterior whole-body planar scans were acquired approximately 2 hours post-injection.

Findings

- Symmetric and physiological tracer distribution is demonstrated throughout the axial and appendicular skeleton. No abnormal focal zones of increased or decreased osteoblastic activity are identified to suggest active metastatic bone disease.
- Mild, non-specific tracer uptake is noted along the spine and major joints, consistent with age-appropriate degenerative changes.
- Normal physiological excretion is visualized through the kidneys and urinary bladder.
- There is a focal area of intense tracer accumulation noted in the left infraclavicular region on the anterior projection. This strictly correlates with the documented site of Port-A-Cath tracer injection and represents local reservoir retention/extravasation. No underlying structural osseous abnormality corresponds to this site.

Impression

- **Negative Bone Scan:** The skeleton demonstrates normal tracer distribution with no scintigraphic evidence of active or cold focal osseous lesions to suggest metastatic bone disease.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/.....

تحية طبية وبعد،

إيماء لطلب المقدم من بشأن تقرير طبي للمريض: عيسى سعود عيسى.

بقسم رقم طبي

نشرّف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد انه:

وزن ٥٨ كغ - ضغط ٩٠/٦٠ - معدل ٩٠

الكبد والطحال طبيعي

عند الفحص

تحريراً في

الوقت: التاريخ: ٢٠١٩/٩/٢٠

الطبيب المعالج

د. محمد كاسبي

يصدق،،